

Watson Healthcare

Employee Attrition Analysis

Identifying the key drivers of turnover and actionable strategies to improve retention

Analysis by S. Jai Varanasi | InsightArc Analytics

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Executive summary

The problem: Watson Healthcare is experiencing an 11.87% attrition rate across 1,676 employees, with 199 departures. While below industry average, the financial and operational impact is significant — each departure costs 0.5-2x annual salary to replace.

The single biggest finding: Overtime is the dominant predictor of attrition. Employees who work overtime leave at 29.2% — nearly 6x the rate of non-overtime workers (5.0%). This creates a vicious cycle: understaffing leads to overtime, which causes burnout, which causes more departures, which creates more understaffing.

The at-risk profile: Young (~31 years old), low-paid (avg \$4,024/month), new to the company (avg 3.7 years), working overtime, with poor work-life balance. Most likely a Nurse or 'Other' role in Cardiology or Maternity.

Recommended actions: Reduce mandatory overtime through additional hiring, increase entry-level compensation, strengthen the first-2-year onboarding experience, and implement nurse-specific retention programs.

Attrition at a glance: 1 in 8 employees is leaving



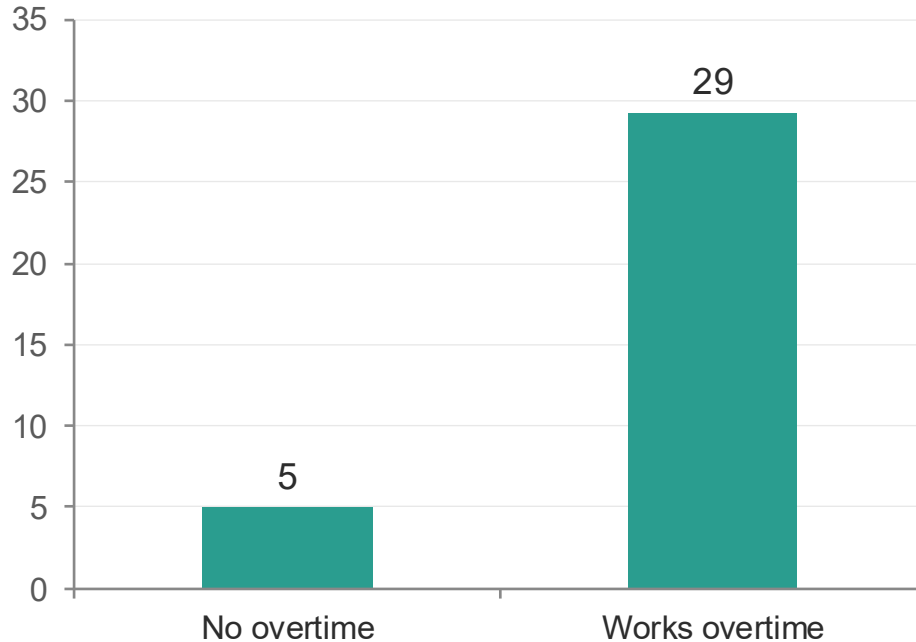
Attrition by department

Cardiology	13.94%	74 of 531
Maternity	12.31%	98 of 796
Neurology	7.74%	27 of 349

Risk tier

- High risk (>15%)
- Moderate risk (10-15%)
- Low risk (<10%)

Overtime is the #1 predictor of attrition — employees who work OT leave at 6x the rate



29.2%

of overtime workers leave

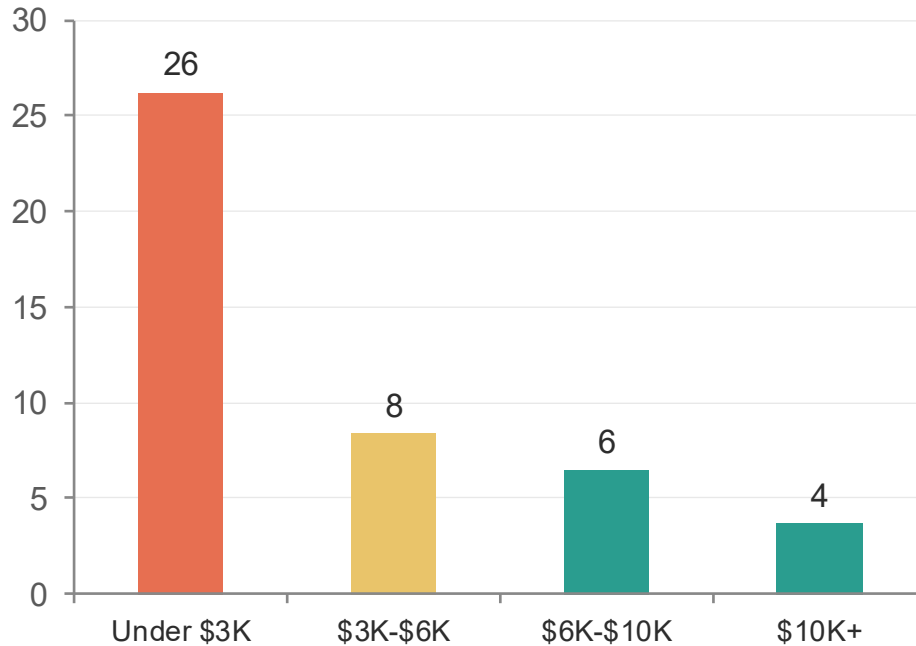
5.0%

of non-overtime workers leave

The vicious cycle: understaffing → overtime → burnout → more departures → more understaffing

Note: Reporting Specific segments of overtime hours worked would give a better picture and allow for more effective policy changes

Lower-paid employees leave at 7x the rate of top earners



Average monthly income

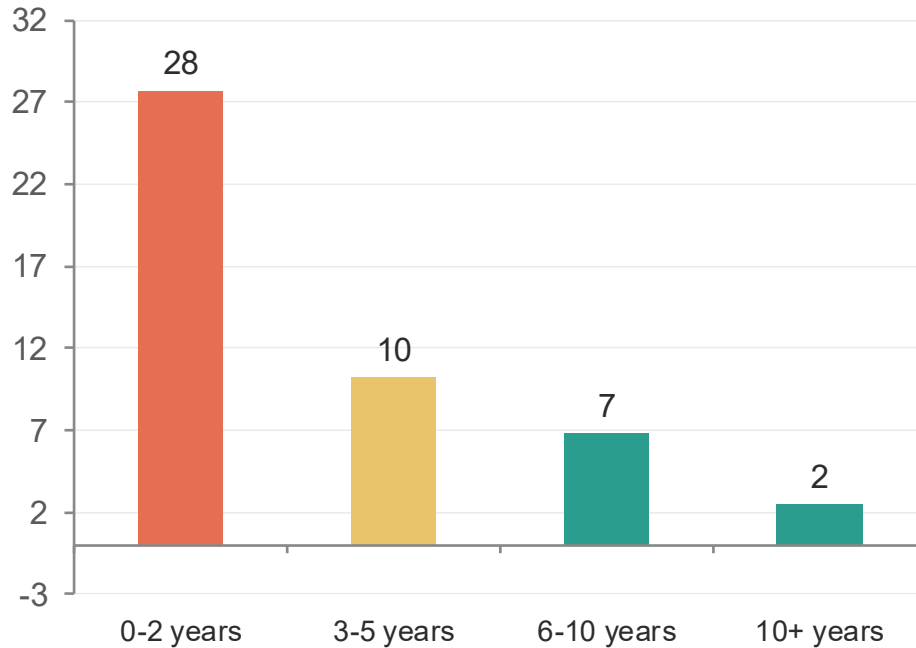
Stayed: **\$6,852**

Left: **\$4,024**

\$2,828 gap

Over 1 in 4 employees earning under \$3K/month leave the organization. Even modest wage increases for the lowest bracket could significantly reduce this 26% attrition rate.

The first 2 years are critical — new hires leave at 11x the rate of tenured staff



107 of 199

departures happen within the first 2 years

Average tenure:

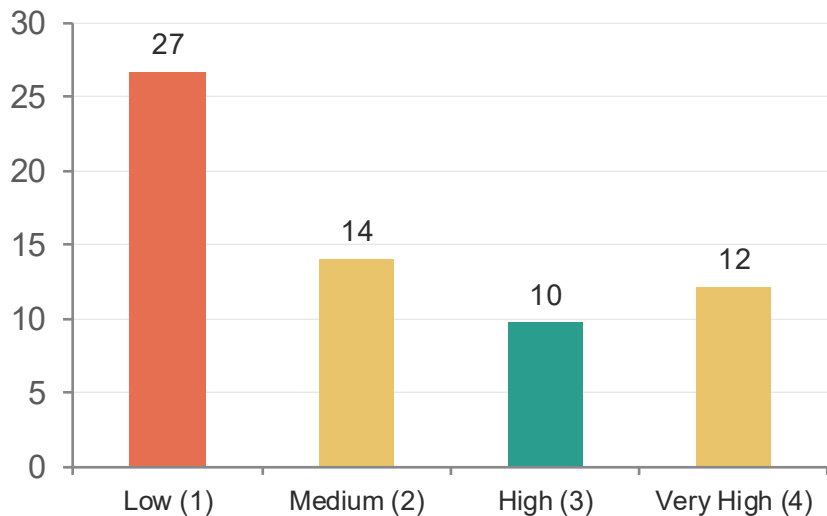
Stayed: 7.48 years

Left: 3.69 years

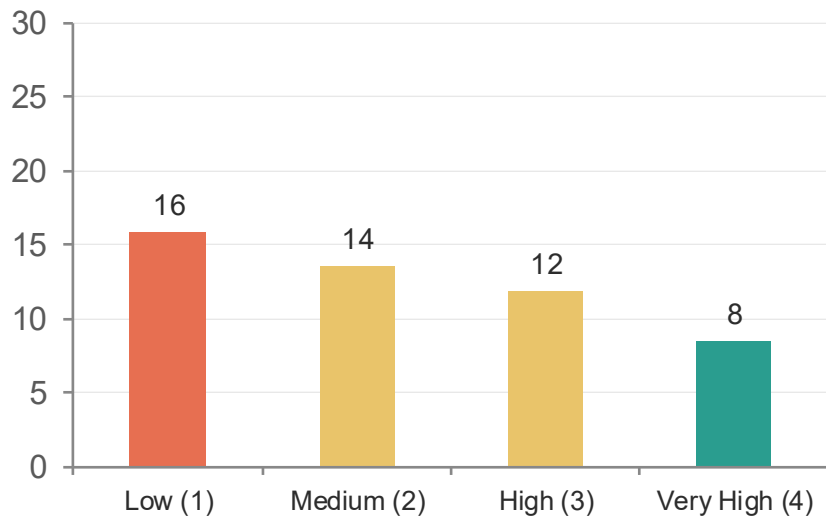
The onboarding window is where retention is won or lost. Structured mentorship, 30-60-90 day check-ins, and buddy systems could dramatically reduce early attrition.

Poor work-life balance and low satisfaction amplify the attrition problem

Work-life balance rating

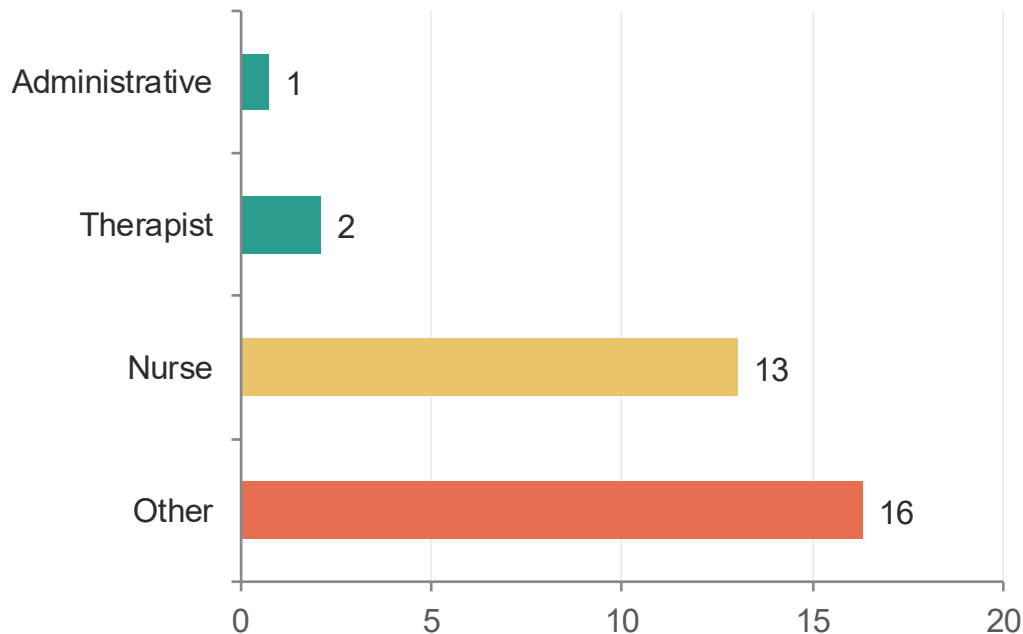


Job satisfaction rating



1 in 4 employees with the lowest work-life balance rating walk out. Satisfaction shows a clear inverse pattern — higher satisfaction consistently reduces attrition across all levels.

Nurses and 'Other' roles drive nearly all turnover — Therapists and Admin rarely leave



822 Nurses

at 13% attrition = largest at-risk group

534 'Other' roles

at 16.3% = highest rate but undefined

Data limitation: 'Other' category needs further breakdown to enable targeted intervention

Five factors account for the vast majority of attrition at Watson Healthcare

01 Overtime

29.2% vs 5.0%

Nearly 6x difference. Single strongest predictor.

02 Low compensation

26.2% for <\$3K

Over 7x the rate of top earners (\$10K+).

03 Short tenure

27.7% in 0-2 yrs

107 of 199 departures in the first 2 years.

04 Work-life balance

26.7% for lowest

Over 2.5x the rate of good balance.

05 Age / career level

Avg age 31

Leavers are younger, lower-level, lower-paid.

Recommendations

Reduce mandatory overtime

Hire additional staff, specifically in Cardiology and Maternity. Address the highest impact factor on the departments with the highest attrition rate

Increase entry-level compensation

Target employees earning under \$3K/month where attrition is 26%. Even modest raises significantly improve retention rate, as the data indicates

Implement a Nursing-Specific Retention Program

Start an incentive program for nurses by offering bonuses, priority scheduling initiatives, etc, to address this group specifically, as this is one of the highest attrition categories

Start enacting policy which would improve work life balance

Flexible scheduling, predictable shift rotations, and adequate time off — especially for roles with lowest balance ratings

Overhaul Onboarding Training Program

Significantly Focus efforts on the early career segment of the employment pool: Mentorship programs, career-ladder programs, etc. Lack of training was a major attrition factor, so implementing a new more robust training program would drive attrition down.